

Female Genitourinary Physical Exam Checklist

Diagnostic Medicine III 2026

STARTING THE PATIENT ENCOUNTER		
Introduction, Verify Patient		
Washes hands and dons appropriate PPE		
General Survey		
Vital Signs (obtain or repeat abnormal vitals)		
EXTERNAL GENITALIA		
Chaperone	Chaperone should be present GU exams: Verbalize: "I will have a chaperone present."	
Consent	Verbalize: "I need to examine your genitalia and rectum. It may be uncomfortable but it should not be painful. Do you have any questions before we begin?"	
Patient Position & Draping	Patient should be positioned in lithotomy position with genital region properly draped. <i>(Sheet should be placed over abdomen and knees)</i>	
Inspection (verbalize)	Pubic hair for:	
	Characteristics	
	Distribution	
	Mons Pubis, Labia Majora, Perineum for:	
	Erythema	
	Masses	
	Lesions	
	Swelling	
	Labia Minora, Clitoris, Urethral Meatus, Introitus (vaginal opening) for:	
	Discharge	
	Erythema	
	Masses	
	Lesions	
	Swelling	
	Verbalize: "If inspection is abnormal, these areas would be palpated"	
Palpation	Bartholin glands if labial mass is seen; palpating for discharge, tenderness and swelling.	
	Perineum for masses and tenderness	
Assessment	Urinary incontinence and vaginal prolapse (ask the patient the bear down)	
INTERNAL GENTILIA		
Insertion of Speculum	Apply lubrication to the speculum using warm water or a small amount of lubrication.	
	Inform the patient that you are going to insert the speculum and will feel your fingers.	
	Separate the labia minora with the nondominant hand. Insert the speculum vertically (widest aspect is vertical to vaginal opening) applying a downward pressure against the perinium.	
	Once the speculum is inserted into vagina, rotate the speculum so that the widest aspect is horizontal.	
	Continue to advance the speculum applying a slight downward pressure <i>(speculum should be inserted far enough that most of the blade is inserted into the vagina)</i> .	
	Slowly open the speculum until the cervix is visualized. Lock the speculum into place.	
Inspection (verbalize)	Cervix for:	
	Color	
	Lesions	
	Erosions	
	Cervical Os for:	
	Bleeding	

	Discharge	
	Shape	
Specimen Collection (PAP Smear)	Collect endocervical cells using the appropriate tools for specimen collection. <i>(Using a PAP smear broom, gently insert the center of the broom into the cervical os, then turn the broom in a circle 3 times.)</i> Once the sample has been collected, drop the broom into the liquid cytology.	
	Collect STI specimen samples using the appropriate cultures.	
Remove Speculum	Unlock the speculum. Gently and slowly withdraw the speculum while keeping the blades partially open.	
Inspection (verbalize)	Vaginal walls while removing speculum:	
	Color	
	Inflammation	
	Discharge	
	Lesions	
	Masses	
BIMANUAL EXAMINATION		
Apply a small amount of lubrication to the index and middle fingers of the dominant hand		
While in the standing position, insert the index and middle fingers of the dominant hand into the vagina. Place the other hand on the abdomen proximal to the pubis.		
Palpation	Vaginal walls for:	
	Muscle tone	
	Tenderness	
	Masses	
	Cervix for:	
	Position	
	Mobility	
	Masses	
	Tenderness	
	Uterus for: (gently apply downward pressure to abdomen)	
	Size	
	Shape	
	Tenderness	
	Position	
	Masses	
	Mobility	
	Consistency	
	Ovaries/Adnexa for: (gently apply a downward sweeping pressure from the right lateral abdomen towards the vagina to palpate the right ovary, and repeat the downward sweeping pressure on the left.)	
	Size	
	Shape	
	Mobility	
	Masses	
	Tenderness	
RECTAL EXAM (for women with a concern or over the age of 40)		
Inspection (verbalize)	Anus for:	
	Lesions	
	Fissures	
	Hemorrhoids	
	Fistulae	
Palpation	Keeping the index finger inserted into the vagina, insert the middle finger into the rectum.	

	Rectovaginal Vault for:	
	Thickness	
	Tone	
	Masses	
	Retrocele (posterior vaginal prolapse - ask patient to bear down while palpating)	
	Remove index finger from the vagina, with the middle finger still inserted in the rectum:	
	Note smoothness and evenness of muscular ring.	
	Slowly rotate finger clockwise direction evaluating rectal walls for:	
	Masses	
	Tenderness	
	Irregularities	
Assess sphincter tone	While finger is inserted in rectum, ask patient to bear down then relax.	
	Withdraw finger and note presence of fecal material for:	
	Color	
	Blood	
	Consistency	
Collection (Verbalize)	When indicated properly collect and prepare specimen.	
Completion of Exam	Remove and dispose gloves.	
	Offer to help the patient sit up/cover up	
	Wash hands with soap and water	